Nursing Evidence- Based Practice

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By

Dr/ Wafaa Osman

Head of mental health nursing department

Out lines:

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- AIMS OF EBP
- Benefit of Evidence Based practice.
- Barrier of Evidence Based practice.
- Hierarchy of Evidence Based practice.
- process of Evidence Based practice.
- Strategies to promote evidence- base practice.

Introduction

There are different sources of knowledge in clinical practice as unsystematic trial and error, tradition, authority, and clinical experience, but not all produce the desired outcomes. So, the process of developing evidence- based nursing practice provides scientific basis for clinical practice which starts with a question about a specific patient problem or situation. Then, a systematic search for evidence could be used to answer this question. Once the evidence is obtained, its validity, reliability, and applicability are appraised. The evidence is then integrated with clinical expertise and patient preferences. Finally, the evidence-based practice decision is implemented and the outcome of the decision is evaluated.

Definitions:

´ **Research** is conducted to generate new knowledge about a particular concern or problem using scientific methods and can be experimental, observational, or descriptive.

´ **Evidence-based practice** is defined as a problem-solving approach to clinical decision-making that integrates the best evidence from research with a clinician's expertise and a patient's personal preferences and values.

´ **Evidence-based nursing** is a process by which nurses make clinical decisions using the best available research evidence, clinical expertise and patient preferences in the context of available resources.

AIMS OF EBP

¢ To provide the high quality and most cast efficient nursing care possible.

¢ To advance quality of care provided b health worker.

¢ To increase satisfaction among patients.

¢ To focus on nursing practice away from habits and tradition to evidence and research.

Benefits of Evidence Based Practice:

1. Ensures that health professionals are delivering patient care that is

innovative and that guaranteeing better outcomes for patients.

2. EBP contributes to the science of nursing and increases the confidence in decision- making, resulting in ideal outcomes.

3. Maintains nursing health practices relevant and current.

4. Reduce costs.

5. Reduce variations in practice.

Barriers of Evidence-Based Practice

1. Research-related barriers:

- Problems in understanding the research language and appraising the quality of evidence.

- Limited number of interventional studies in nursing clinical practice.

Barriers to Evidence-Based Practice

2. Nurses related barriers:

- Lack of knowledge and skills (about the research process and how to critique research studies).

- Insufficient time to find research reports.

- Lack of resources.

Barriers to Evidence-Based Practice

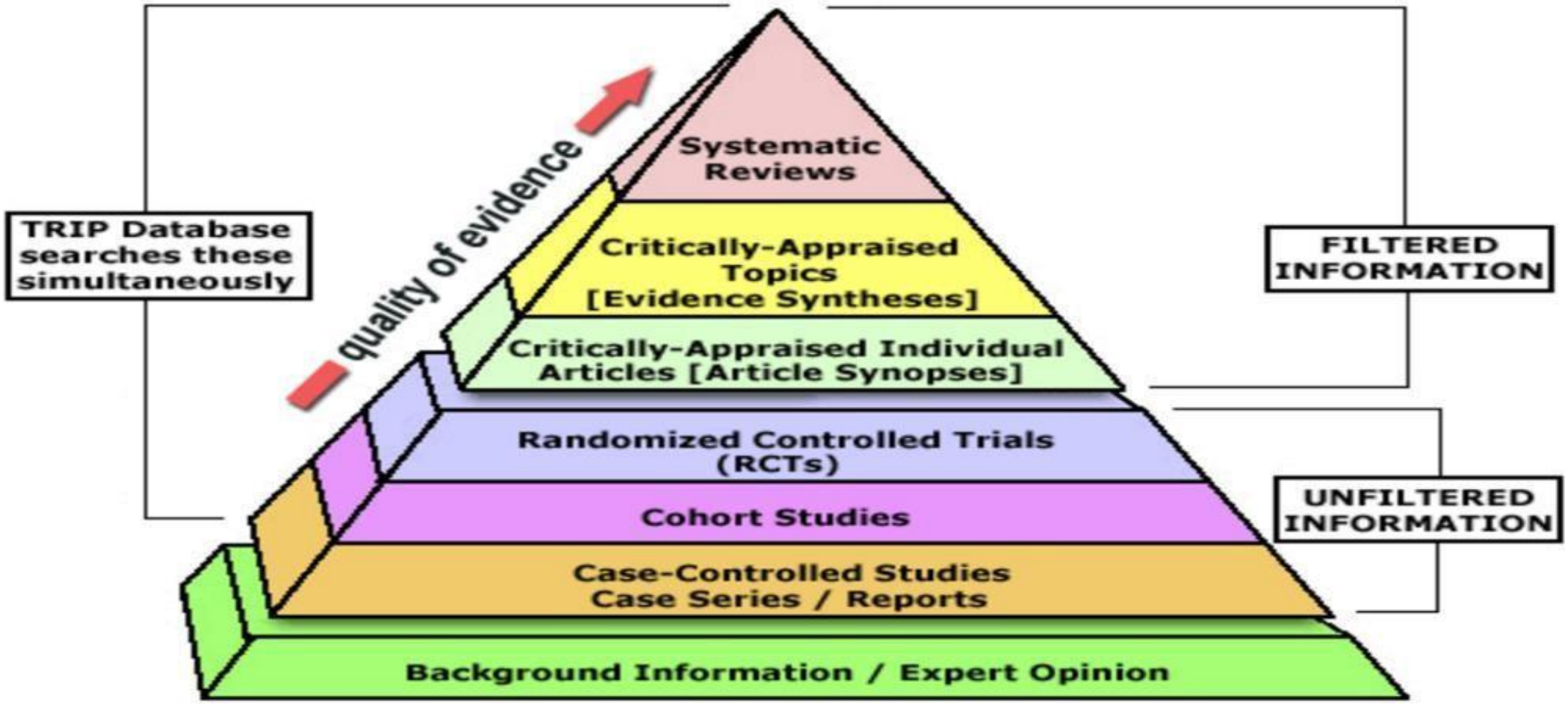
3. Organizational barriers:

- An organizational culture that doesn‘t support EBP

- Heavy workload.

- Lack of access to nursing journals & database.

Hierarchy of EBP



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What is the best available evidence?

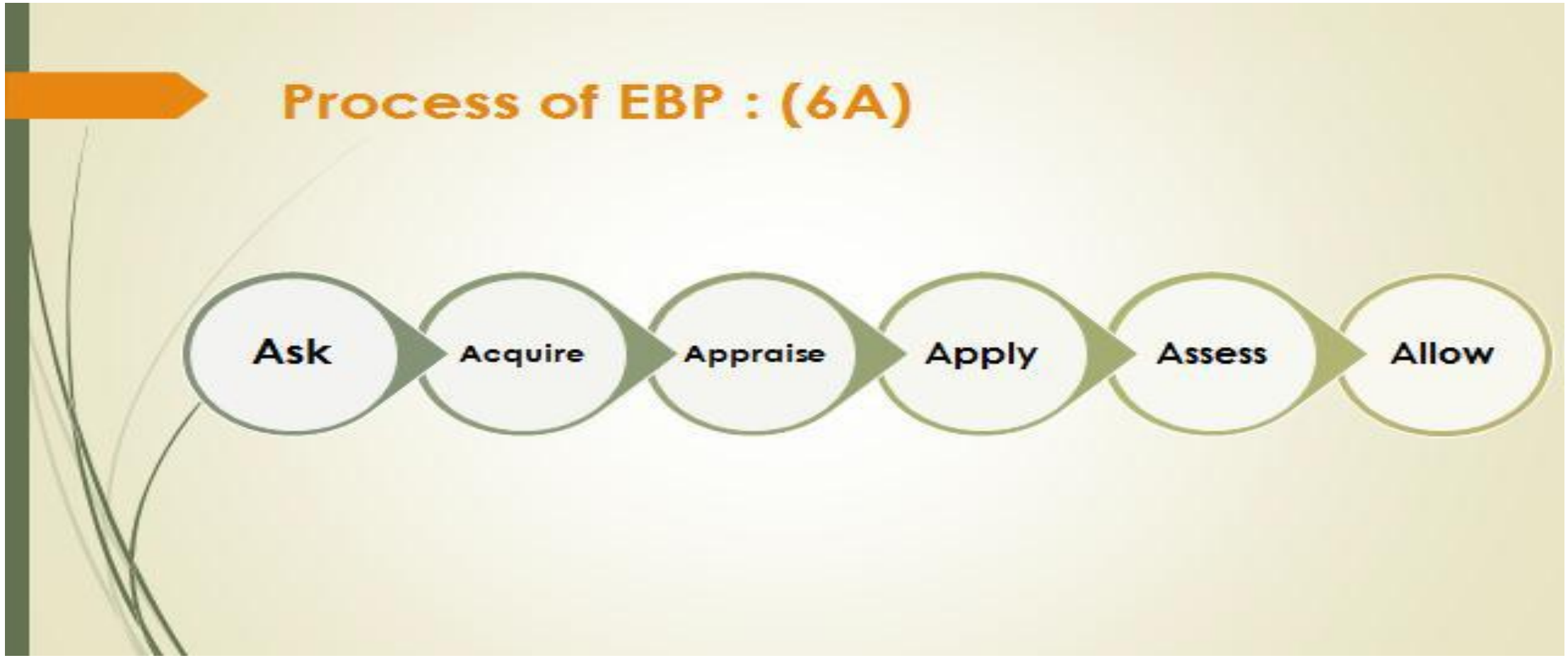
- Is a core principle of (EBP) and attempts to address this question.

EBP hierarchies rank study types based on the rigor (strength and precision) of their research methods.

Different hierarchies exist for different question types

- Most experts agree that the higher up the hierarchy the study design is positioned
- In most evidence hierarchies current, well-designed systematic reviews and meta-analyses are at the top of the pyramid, and expert opinion and anecdotal experience are at the bottom.

The process of Evidence Based Practice (6A) :



Steps of Evidence-based nursing process

Step1:

Asking :(translating a practical issue or problem into an answerable question).

The question should be in PICOT or PICO format (used to break down the question into smaller parts and identify keywords).

PICOT

P= Patient / Population

Who are the relevant patients?

age, sex, geographic location, or specific characteristics that would be important to question.

C= Compare /Control

Is there a control or alternative management strategy you would like to compare to the intervention or indicator?

I= Intervention / Indicator

What is the management strategy, diagnostic test, or exposure that you are interested in?

O= Outcome

What is the patient-relevant consequences of the intervention?

T= Time / Type of Study or Question

what periods should be considered?
What study types are most likely to
Have the information you seek?
What clinical domain does the question fall under?

Step (2) A quire:

A quire:
(systematically searching for and retrieving the evidence).

Search and select the appropriate resources to find the best evidence to answer the questions.

The resources divided into 3 categories:

· General information (background) resources.

· Filtered resources.

· Unfiltered resources.

✓ General information (background) resources

This category contains resources that provide background information about various clinical questions.

Resources include up to- date and other E-Books , textbook format, journal literature.

- ✓ **Filtered resources** are secondary sources that summarize and analyze the available evidence.

filtered resources include Cochrane Database of Systematic Reviews, Guidelines International Network.

- ✓ **Unfiltered resources** are primary or original sources that provide evidence concerning a topic under investigation. unfiltered resources include, Web of Science, Scopus and PubMed.

Step (3)
Appraise :

Appraise (critically judging the trustworthiness and relevance of the evidence)

- Critical appraisal is the process of systematically examining research evidence to assess its validity, results, and relevance before using it to inform a decision.

Techniques for critical appraisal:

✓ **Validity (is the study valid)?**

validity is a way to assess the quality of a research study. Validity refers to

how accurately a method measures, what it is intended to measure.

Relevance (How will the results help working with the patient?)

The research covered the outcome for patients and the outcomes are ptients oriented or disease oriented.

The results (What are the results?).

- Reading the results aims at deciding whether it is worth to change to the new modality of treatment or not.

Step (4) Apply *:*

Apply: (incorporating the evidence into the decision-making process)

To achieve a measurable improvement when the implementation

criteria are met:

□ Quality of patient care

□ Consistency of patient care

□ Patient outcomes

□ Cost containment

Step (5)
Assess :

Assess (evaluating the outcome of the decision taken).

The fifth step of the process, outcome evaluation, attempts to interpret the results and evaluate the outcomes of the applied evidence (intervention).

Depending on the type of outcomes achieved, it may be possible to compare the outcomes of a study with similar outcomes on a local, regional, national, or international level.

Step (6)
Allow:

- **Allow:** Dissemination the outcomes of the EBP decision or change through presentation or publication.

Example:

- In patients with schizophrenia (**Population**),
- what is the effect of art therapy (**Intervention**)
- compared to antipsychotic medications (**Comparison**) in controlling positive symptoms of schizophrenia
- within the first month of treatment (**Time**)?

Strategies to promote evidence- base practice:

The organization's readiness to receive and use evidence.

- Education and training.
- The availability of access to supportive resources and tools.
- Interactions between researchers, research users and other stakeholders.
- Patient engagement in EBP
- Continuous learning, research & innovation

Critical thinking question ?

THANK YOU